28 October 2026 | Evaluation plan v6 | Discussion draft — no expenditure approval
MEMORANDUM

Panda Vision: explicit pathways and investment evaluation

Action required

Use the repaired existing-evidence model for conditional investment analysis. Compare five complete consultation configurations while distinguishing economic resources, actual budgets and funding. Do not treat estimated savings, an unquoted prototype or software tests as clinical validation. Independent review and critical full-text verification remain incomplete.

Author	Prepared for Dr Dylan Mordaunt, Chief Medical Officer
Presented by	CMO / neonatal, retrieval, virtual-care and finance partners

Executive summary

1. Decision and scope. Compare telephone-supported consultation, mobile teleconference, Panda Vision, fixed ceiling/wall PTZ and a complete tablet/OBSBOT kit. Retain specialist consultation and RSQ/ANTS-NQ escalation in every arm. Distinguish new investment, existing equipment and shared use. Pilot activity remains 10–15 consultations per site/year at Atherton, Mareeba and Innisfail; 4–5 Atherton cases do not estimate effectiveness. [ELC2026; USER2026]

2. Explicit clinical pathways. For each severity stratum, show immediate transfer, initial local care followed by delayed rescue, and final local care. The primary hypothesis applies the video odds ratio to eventual transfer, then partitions timing; it does not penalise the same effect twice. Failed video follows telephone care. Test an initial-decision interpretation, increased delay and excess rescue separately. No mortality benefit is assumed. [M6; HAY2020]

3. Hospital and hub resources. Price originating care, receiving care, final local care and additional pre-rescue care once each. Unadjusted IHACPA episode prices provide provisional prototypes, not patient-level coding or marginal cash costs. Separate hospital and transport avoidable fractions. Replace the site-count hub trigger with incremental video minutes × loaded hourly cost; additional paid capacity depends on workload exceeding explicit spare capacity. Existing telephone time is not recharged. [NEP26; M6]

4. Complete and interpret the analysis. The repaired kernel has 22 paired 20,000-draw structures, 12 one-way dimensions, three two-way grids, a 625-point four-way grid and seven-year cash/replacement schedules. Clinical effects and systematic costs remain shared across strategies/sites. The pilot reference gives 50.5% probability of resource saving for Panda versus telephone; the illustrative state figure is 65.6%. These are changed model estimates, not new empirical evidence. [M6]

5. Verify evidence and validity. Full article bodies were reviewed for Lietz, Maddox and the Fox transport-cost paper. The original Haynes transfer study and Armfield/Yoo/Thao economic papers remain full-text access gaps. Preserve their provisional status, null/harm scenarios and transportability caveats. A separate scalar arithmetic implementation and conservation tests passed; no independent subagent panel could be executed. Four reviewer briefs and a pending-status register are provided. [E6; V6]

6. Prepare the publication. Lead with the investment question, explicit pathways, source/assumption table, paired uncertainty and decision thresholds. Move machine learning and speculative lifetime results out of the main argument. Use ISPOR–SMDM/AdViSHE/TECH-VER status documentation and CHEERS reporting, without claiming certification. Complete independent face validation, full-text gaps and a clean locked-environment run before submission. No RCT, new recruitment or structured elicitation is a prerequisite. [V6]

Engagement and consultation	
Consumers	Describe family expenditure and separation as a limited extension, not a complete societal evaluation.
Clinicians and staff	Seek independent clinical/economic scrutiny of the specified pathways and provisional costs; no numerical elicitation is required.
External stakeholders	No external, neonatal-network or vendor endorsement is implied. Subagent briefs are unexecuted, not independent reports.

Implications	
Budget	Use annual undiscounted cash schedules. Include future replacement of installed devices, incremental capacity and local running costs. Do not add NWAU × NEP or QWAU × QEP to resource costs; current QEP remains unverified.
FTE	Hub reference assumptions: 15 extra video minutes/call at $200/hour; $25 local coordination/call. Capacity scenario: 100 spare hours, then 200-hour/$40,000 blocks. These are explicit estimates, not an establishment request.
Risk	Clinical-effect transportability, timing, hospital-price mapping and local resource fractions remain uncertain. Equal device effectiveness is a hypothesis; a cheaper kit is not proven clinically equivalent.
Legal	Existing aggregate-evidence modelling. Obtain institutional governance, authorship and data-rights decisions before publication. No ethics exemption or clinician sign-off is invented.
ICT	Installation estimates and complete prototype cost require verification. Biomedical, infection-control, cyber, power, mounting and far-end-control eligibility remain separate from economic ranking.
Impact on First People's Health	Assess access and displacement separately. Do not apply a cost-only rule to clinical care or exclude additional survivors with morbidity. Lifetime health gains remain unquantified in the primary analysis.

Strategic Plan alignment
Our care: appropriate neonatal support. Our people: usable systems and explicit workload. First Peoples health: equitable access. Our sustainability: transparent resources, budgets and uncertainty.

Attachment/s
1. v6 methods/results supplement. 2. Updated tables, figures and results workbook. 3. CHHHS presentation. 4. Versioned repository, source verification register, validation status and reviewer briefs. M6: model outputs; E6: evidence audit; V6: validation record. Historical v5 results are retained but superseded for current decisions.